

Gender: Female

Race: White

UUID:1FC2AC75-8248-4284-82C6-D664FA8DEB0A
TCGA-FD-ASBZ-01A-PR

Redacted



Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

- A. Left pelvic lymph nodes:
 - Fourteen lymph nodes, negative for tumor (0/14).
- B. Right pelvic lymph node:
 - Fifteen lymph nodes, negative for tumor (0/15).
- C. Bladder:
 - Urothelial carcinoma, invasive, high grade, with focal squamous differentiation. < 10% pTSS
 - One lymph node with microscopic focus of urothelial carcinoma (1/1).
- D. Uterus bilateral tubes and ovaries:
 - Atrophic endometrium.
 - Myometrium with three (3) leiomyomas (largest posterior wall, 1.0 cm).
 - Atrophic ovaries and unremarkable fallopian tubes.
- E. Right distal ureter tip:
 - Ureter, no evidence of tumor.

Urothelial Carcinoma Pathologic Parameters

- 1. Tumor type: Flat urothelial carcinoma
- 2. Grade of tumor: High grade
- 3. Depth of invasion: Extravesicular soft tissue
- 4. Tumor distribution:
 - Multifocal, 3.2 x 1.8 largest size
 - Anterior wall
- 5. Ureteral Margins: Negative
- 6. Distal urethral margin: Negative
- 7. Lymph nodes: Negative (1/30)
- 8. pTNM: pT3a, N1, MX

ICD-O-3

Carcinoma, Urothelial NOS
8120/3

Site: Bladder, Anterior
C67.3

JW 12/24/12

xxx

[] M.D.

Interpretation performed by the Attending Pathologist and reviewed with the Resident or Fellow.

Electronically Signed Out by [], M.D.

Clinical History:

The patient is a year old female with bladder cancer who undergoes radical cystectomy with ileal conduit.

Specimens Received:

- A: Left pelvic lymph nodes
- B: Right pelvic lymph node
- C: Bladder
- D: Uterus bilateral tubes and ovaries
- E: Right distal ureter tip

Gross Description:

The specimen is received in five containers each labeled with the patient's name and medical record number.

A. Container A is further designated as 31. Left pelvic lymph nodes.⁴ Received fresh and placed in formalin is an 8 x 5.2 x 1.9 cm aggregate of yellow, lobulated adipose tissue which is dissected for putative lymph nodes. Representative sections are submitted as follows:

Block Summary:

A1: five putative lymph nodes
A2: two putative lymph nodes, bisected (one inked black)
A3: two putative lymph nodes, bisected (one inked black)
A4-A6: one putative lymph node, trisected, in each cassette
A7: one putative lymph node, bisected
A8-A11: one putative lymph node in four pieces
A12-A14: one putative in three pieces

B. Container B is further designated as 32. Right pelvic lymph nodes.⁴ Received fresh and placed in formalin is a 6 x 6 x 1.2 cm aggregate of yellow, lobulated adipose tissue which is dissected for putative lymph nodes. Representative sections are submitted as follows:

B1-B2: four lymph nodes, each cassette
B3-B4: two lymph nodes, bisected each cassette (one inked black)
B5: one lymph node, trisected in multiple sections
B6-B8: one lymph node, trisected

C. Container C is further designated as 33. bladder.⁴ Received fresh and placed in formalin is a 191 gram, 15.5 x 9.5 x 4.5 cm cystectomy specimen with attached mesenteric fat. The bladder proper measures 7.5 x 7.5 x 4 cm, with a 3 x 2.2 cm urethra. The right ureter measures 4.3 x 0.5 cm and the left ureter measures 4 x 0.5 cm. The external surface of the bladder is inked in black and the bladder is opened along the lateral wall to reveal an ulcerated lesion on the anterior wall measuring 3.2 x 1.8 cm. In addition, there is a 2.5 x 1.3 cm transmural defect in the anterior wall at the level of trigone. The ulcerated lesion consists of glistening, brown finely nodular and friable tissue. The remaining bladder mucosa is unremarkable, glistening, pink-tan with normal mucosal folds. Serial sectioning reveals tumor invasion to a depth of 1.0 cm, extending to 0.3 cm from the inked margin. Gross photographs are taken, representative sections are submitted as follows:

C1: urethral margin
C2: right ureter margin
C3: left ureter margin
C4- C7: multiple representative sections through the tumor including closest extent to black, inked margin
C8: representative section of grossly uninvolved anterior wall
C9: representative uninvolved section of posterior wall
C10: representative sections of uninvolved right and left bladder wall
C11: representative section of bladder dome
C12: representative section of trigone
C13: two putative lymph nodes
C14: right ureter margin
C15: left ureter margin

D. Container D is further designated as 34. Uterus bilateral tubes and ovaries.⁴ Received fresh and placed in formalin is a 40 gram supracervical hysterectomy specimen consisting of a uterus with bilateral attached tubes and ovaries. The uterus measures 4 cm from cornu to cornu, 3.5 cm from fundus to lower uterine segment and 2 cm from anterior to posterior. The cauterized resection margin measures 2.5 x 1.2 cm. The left tube measures 5 x 0.5 cm and the left ovary measures 1.5 x 1.5 x 0.5 cm. The right tube measures 7 x 0.5 cm and the right ovary measures 2 x 1 x 0.5 cm. The uterine serosa is glistening, pink-tan without nodules, however on the anterior and posterior aspect of the uterus there are 1 cm linear serosal defects consistent with surgical manipulation. The posterior aspect of the uterine serosa is inked in black and

the specimen is opened to reveal a triangular shaped endometrial cavity measuring 2.5 x 1 cm with a thickness of 0.1 cm. The uterine myometrium measures up to 0.9 cm thick and is trabeculated pink-tan. The uterus is serially sectioned revealing a subserosal posterior wall firm, white nodule measuring 1 cm in greatest dimension and an intramuscular anterior wall firm, white nodule measuring 0.2 cm in greatest dimension. Both nodules are devoid of hemorrhage or necrosis. The left ovary has a glistening, pink-white surface. It is sectioned to reveal homogeneous, pink-white cut surface. The tube is serially sectioned revealing a pinpoint lumen and wall thickness of 0.1 cm. The surface of the right ovary is glistening, pink-white. It is serially sectioned to reveal a homogeneous pink-white cut surface. The tube serosa is glistening, pink-gray and the tube is serially sectioned revealing a pinpoint lumen and a 0.1 cm wall thickness. Representative sections are submitted as follows:

- D1: representative posterior endomyometrium
- D2: representative posterior endomyometrium including nodule
- D3: representative anterior endomyometrium including nodule
- D4: representative left ovary
- D5: representative left tube including fimbriated end
- D6: representative right ovary
- D7: representative right tube including fimbriated end
- D8: representative posterior wall surgical margin
- D9: representative anterior wall surgical margin

E. Container E is further designated as 35. Right distal ureter tip.4 Received in formalin is a 1.5 cm long x 0.4 cm in diameter piece of tubular tissue with a scant amount of attached yellow, lobulated adipose tissue. The tubular tissue is rubbery with a glistening, pink-white surface and is patent at both ends. One end has a clear, surgical suture. The specimen is serially sectioned revealing a pinpoint lumen and a 0.1 cm wall thickness. It is entirely submitted as follows:

- E1: margin designated with suture
- E2: entire remaining specimen

xxx
[], M.D. Ph.

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	KMI	Date Reviewed: 12/18/12